FOR EDITH · PAEDIATRICS & CHILD HEALTH JUNIOR CLERKSHIP

Acute Lower Respiratory Tract Infections

Tutorial | Week 1 · Breathing, fever & the clinical methods | Paediatrics | Dr Nabatte

Pneumonia is the biggest infectious killer of Ugandan children; it is diagnosed by counting breaths, looking for chest indrawing and danger signs, and measuring SpO_2, then classified by WHO/IMCI into pneumonia (oral amoxicillin at home) or severe pneumonia (oxygen, ampicillin or penicillin plus gentamicin in hospital), while bronchiolitis, pertussis, empyema, aspiration and HIV-related pneumonia each need their own recognition and treatment.

Must know before the ward round

1. Fast breathing (WHO/IMCI): ≥60/min under 2 months; ≥50/min at 2–11 months; ≥40/min at 1–4 years; UCG 2023: >30/min from 5 years. Count for a full 60 seconds in a calm child.
2. Pneumonia (WHO 2014/UCG 2023/WHO 2024) = cough or difficult breathing + fast breathing and/or chest indrawing, no danger sign → oral amoxicillin 40 mg/kg twice daily (80 mg/kg/day) for 5 days at home (3 days only for fast-breathing pneumonia in low-HIV areas; Uganda is high-HIV, so 5 days).
3. Severe pneumonia = cough/difficult breathing + any general danger sign (unable to drink, vomits everything, convulsions, lethargic/unconscious), central cyanosis or SpO_2 <90%, or severe respiratory distress (grunting, very severe indrawing) → admit, oxygen, ampicillin 50 mg/kg (or benzylpenicillin 50,000 IU/kg) IV/IM 6-hourly + gentamicin 7.5 mg/kg once daily for at least 5 days.
4. Every infant under 2 months with pneumonia is treated as severe (UCG 2023): ampicillin + gentamicin (5 mg/kg in the first week of life).
5. No better after 48 h on first line → ceftriaxone 80 mg/kg once daily; suspect Staph (pneumatocoeles, empyema) → cloxacillin 50 mg/kg 6-hourly + gentamicin for 3 weeks; think TB, HIV/PCP, empyema, foreign body, heart failure.
6. Oxygen for SpO_2 <90% (WHO); target ≥94% if emergency signs. Nasal prongs 0.5–1 L/min neonates, 1–2 L/min infants, 2–4 L/min older children. Without an oximeter: central cyanosis, grunting, head nodding, nasal flaring, inability to drink, RR ≥70 suggest hypoxaemia.
7. Bronchiolitis: under 2 years (peak 2–6 months), RSV, coryza then cough, fast breathing, crackles and wheeze. Supportive care only: oxygen if SpO_2 <90–92%, small frequent or NG feeds, gentle nasal suction. No routine antibiotics, steroids or nebulised therapy (NICE NG9); UCG 2023 allows a salbutamol trial, stopped if no response.
8. HIV-exposed/infected infant with severe pneumonia: add high-dose cotrimoxazole for PCP (UCG: 120 mg/kg/day in 3–4 doses for 21 days) + prednisolone if hypoxic; start or continue ART.
9. Pertussis: catarrhal → paroxysmal (whoop, post-tussive vomiting, apnoea and cyanosis in infants) → convalescent; azithromycin 10 mg/kg (day 1, then 5 mg/kg days 2–5 if ≥6 months; 10 mg/kg daily for 5 days in infants <6 months) or erythromycin 12.5 mg/kg 6-hourly; treat contacts; DTP at 6, 10, 14 weeks.
10. Empyema: persistent fever on antibiotics, stony dull percussion, absent breath sounds → CXR/ultrasound, diagnostic tap, chest drain, cloxacillin + gentamicin (Staph) for 3–4 weeks.
11. Kerosene/paraffin ingestion: never induce vomiting or wash out the stomach; give oxygen, observe at least 6 h, CXR; antibiotics only if secondary infection.
12. Pneumonia killed about 740,000 under-fives in 2019 (WHO); pneumonia and diarrhoea together cause about 23% of under-five deaths (WHO 2024).

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1. Definition

Lower respiratory tract infections (LRTIs) are infections below the larynx: **pneumonia** (infection of the lung parenchyma), **bronchiolitis** (viral inflammation of the small airways in infants), **bronchitis**, and the special cases of **pertussis**, **empyema**, **aspiration** and **HIV-related pneumonia** (PCP). Pneumonia is the **single largest infectious cause of death in children worldwide** (WHO fact sheet), and in Uganda it is one of the top three killers of under-fives, alongside malaria and neonatal conditions.

The WHO/IMCI approach was designed so that a health worker with a watch and a pair of eyes can save most of these children:

1. **Count** the breaths for a full minute, **look** for chest indrawing, **check** for danger signs, and **measure SpO_2**.
2. **Classify**: no pneumonia (cough or cold) / **pneumonia** (oral amoxicillin at home) / **severe pneumonia** (refer, oxygen, injectable antibiotics).
3. **Give oxygen** when SpO_2 is below 90%. Hypoxaemia is the strongest predictor of death, and oxygen is the most life-saving drug on the ward.
4. **Review at 48 hours** and think of the reasons for failure: Staph, TB, HIV/PCP, empyema, foreign body, heart failure, malnutrition.

> **CLINICAL PEARL**
> The one thing never to forget: **put a pulse oximeter on every child with cough or difficult breathing.** Clinical signs miss about a third of hypoxaemic children, and a child with SpO_2 <90% is several times more likely to die. Oxygen is cheap; a missed reading is not.

Term	Meaning
Pneumonia	Infection of the lung parenchyma (alveoli). Clinically (WHO): cough or difficult breathing **plus fast breathing and/or chest indrawing**
Fast breathing (WHO/IMCI)	**<2 months: ≥60/min; 2–11 months: ≥50/min; 12–59 months: ≥40/min**; ≥5 years: >30/min (UCG 2023)
Lower chest wall indrawing	The lower chest wall goes **in** when the child breathes **in** (not just the soft tissue between the ribs); must be present all the time and in a calm child
General danger signs (IMCI)	**Unable to drink or breastfeed, vomits everything, convulsions (now or in this illness), lethargic or unconscious**
Hypoxaemia	SpO_2 **<90%** at sea level (<87% above 2,500 m)
Bronchiolitis	Acute viral LRTI in infants <2 years: coryzal prodrome, then cough, tachypnoea or recession and wheeze and/or crackles (NICE NG9)
Empyema	Pus in the pleural space
Pneumatocoele	Thin-walled air cyst in the lung, typical of **staphylococcal** pneumonia

Term	Meaning
Aspiration pneumonitis / pneumonia	Lung injury from inhaled gastric contents, hydrocarbons (kerosene) or oropharyngeal secretions; infection may follow
Pertussis (whooping cough)	Toxin-mediated respiratory infection with *Bordetella pertussis*: paroxysmal cough with whoop, vomiting, apnoea in infants

2. Epidemiology

- **Global**: pneumonia accounted for **14% of under-five deaths (740,180 children in 2019)** and 22% of deaths in children aged 1–5 years (WHO fact sheet 2022). Pneumonia and diarrhoea together cause **23% of under-five deaths, about 1.17 million a year** (WHO 2024 guideline).
- **Uganda**: one of the top three killers of under-fives, with malaria and neonatal conditions.
- **HIV-infected infants**: *Pneumocystis jirovecii* causes at least a quarter of pneumonia deaths (WHO).

What is available

Level	Usually available	Often missing
VHT (iCCM)	Respiratory timer, **amoxicillin DT**, referral	Oximeter (some now have)
HC III	Amoxicillin DT, first-dose injectable (benzylpenicillin, gentamicin), paracetamol	Oxygen, oximeter (improving), X-ray
HC IV / General Hospital	Oxygen concentrators, pulse oximeters, ampicillin, gentamicin, ceftriaxone, cloxacillin, X-ray (some)	Reliable power for concentrators, blood culture, chest drains
Regional / National Referral	X-ray, ultrasound, blood culture, chest drains, bubble CPAP (Mulago, some RRHs), HDU	PICU and ventilators (very few beds)

UGANDA CONTEXT

Oxygen access has improved in Uganda through concentrators, cylinder plants at regional referral hospitals and **solar-powered oxygen** projects, but many wards still run out at night or during power cuts. Check the concentrator and the oximeter at the start of every shift, and **prioritise oxygen** for children with SpO_2 <90%. Bubble CPAP made from nasal prongs, tubing and a water bottle is used at Mulago and several regional hospitals for severe pneumonia and bronchiolitis.

3. Aetiology and Risk Factors

Causes by age (UCG 2023; WHO)

Group	Organisms
Neonates	Group B streptococcus, *Klebsiella*, *E. coli*, *S. aureus*, *Chlamydia*
Children <5 years	*Streptococcus pneumoniae* (commonest bacterial), *Haemophilus influenzae*, *S. aureus*; viruses (**RSV** the commonest viral cause, influenza, measles, hMPV)
≥5 years	Pneumococcus, *Mycoplasma pneumoniae*, viruses
HIV-infected infants	*Pneumocystis jirovecii*, CMV, TB, Gram negatives
Bronchiolitis	RSV
Pertussis	*Bordetella pertussis*

Group	Organisms
Staphylococcal pneumonia	Follows measles, influenza, skin sepsis or pyomyositis

Risk factors: malnutrition (wasting, stunting), HIV infection or exposure, no breastfeeding in the first 6 months, low birth weight, **indoor air pollution from charcoal and firewood**, crowding, parental smoking, missed vaccines (PCV, Hib, measles, pertussis), measles, sickle cell disease.

Aspiration risk: cerebral palsy, reduced consciousness, swallowing problems; toddlers eating groundnuts, beans or maize (foreign body); kerosene or paraffin stored in drink bottles.

4. Pathophysiology

How pneumonia makes a child hypoxic

1. Organisms reach the alveoli (aspiration of nasopharyngeal flora, inhaled droplets, or blood spread in Staph).
2. **Inflammation fills alveoli with fluid, neutrophils and fibrin** (consolidation).
3. **Ventilation–perfusion mismatch and shunt**: blood passes alveoli that are not ventilated → **hypoxaemia** (not corrected by breathing faster).
4. The child compensates by **breathing faster** (tachypnoea) and harder (**indrawing**, nasal flaring, grunting = breathing out against a partly closed glottis to keep alveoli open; head nodding = accessory neck muscles in an infant).
5. Fatigue, sepsis, dehydration, hypoglycaemia and **hypoxaemia** cause death; malnutrition and HIV blunt the signs (less fever, less tachypnoea).

Why the signs work

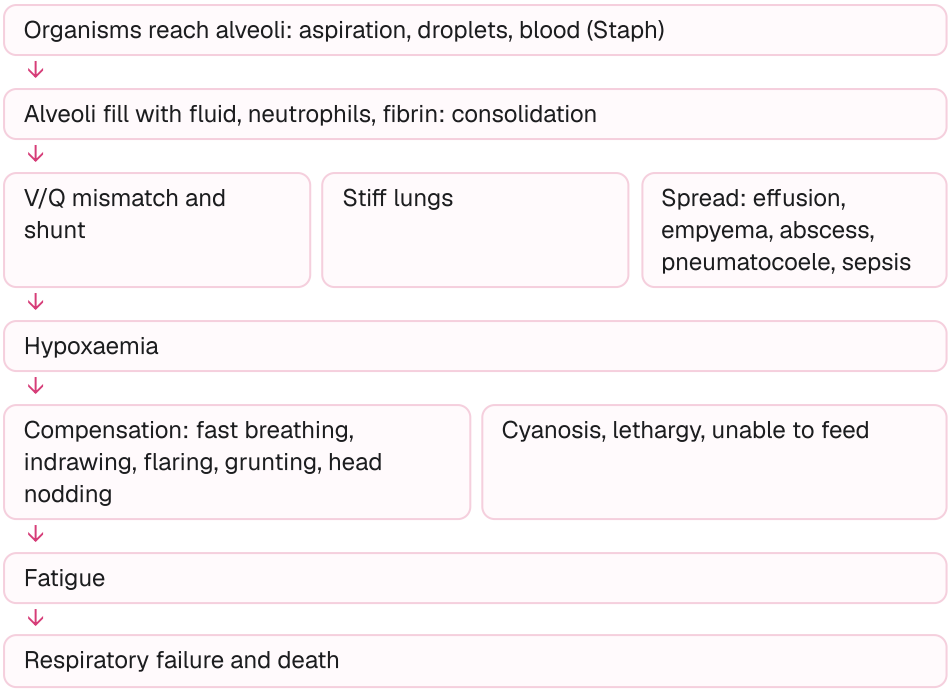
- **Fast breathing** is the most sensitive single sign of pneumonia; age-specific cut-offs account for normal infant rates.
- **Chest indrawing** reflects stiff (non-compliant) lungs pulling on a compliant chest wall; it marks more severe disease.
- **Crackles** come from fluid in small airways and alveoli; **bronchial breathing** from consolidation transmitting tracheal sounds; but auscultation in a crying infant is unreliable, which is why WHO relies on counting and looking.

Bronchiolitis: RSV infects the bronchiolar epithelium → **necrosis, oedema, mucus and cell debris plug the small airways** → air trapping (hyperinflation, wheeze) and atelectasis (crackles) → V/Q mismatch and hypoxaemia. Infants have tiny airways and are obligate nasal breathers, so a blocked nose worsens feeding and breathing. The illness **peaks on days 3–5** and cough resolves within 3 weeks in 90% (NICE NG9). β_2-agonists don't work because the problem is plugging, not smooth-muscle spasm.

Pertussis: *B. pertussis* attaches to ciliated epithelium and releases **pertussis toxin** (lymphocytosis, hypoglycaemia, impaired immune response) and tracheal cytotoxin (ciliary damage). Thick mucus and damaged cilia cause **paroxysms** of coughing; the inspiratory **whoop** is air rushing through a narrowed glottis after the paroxysm. Young infants may not whoop: they **stop breathing (apnoea) and go blue**.

Staphylococcal pneumonia and empyema: *S. aureus* produces toxins (e.g. Panton-Valentine leukocidin) that cause **necrosis**: abscesses, **pneumatocoeles**, **pneumothorax** and **empyema**. It often follows measles, influenza, skin sepsis or pyomyositis, and progresses fast.

Hydrocarbon aspiration: kerosene and paraffin have **low viscosity and surface tension**: tiny amounts spread over the airways, dissolve surfactant and cause chemical pneumonitis. Inducing vomiting gives the fluid a second chance to be aspirated.



Feature links

- Alveolar exudate → V/Q mismatch and shunt → hypoxaemia → low SpO_2, central cyanosis, restlessness, lethargy.
- Stiff, consolidated lungs → small tidal volume → fast breathing.
- Stiff lungs pulling on a compliant chest wall → lower chest wall indrawing.
- Expiration against a partly closed glottis to keep alveoli open → grunting.
- Accessory neck muscles in an infant → head nodding; increased work → nasal flaring.
- Work of breathing → cannot drink or breastfeed → dehydration, hypoglycaemia.
- Fluid in small airways → crackles; consolidation transmitting tracheal sounds → bronchial breathing, dullness.
- Pus in the pleural space → stony dullness, absent breath sounds, mediastinal shift, persistent fever.
- Staph necrosis → pneumatocoeles, abscess, pneumothorax (sudden deterioration, hyper-resonance).
- Bronchiolar plugging → air trapping → wheeze, hyperinflation; atelectasis → fine crackles.
- Blocked nose in an obligate nasal breather → poor feeding and worse distress.
- Pertussis toxin → lymphocytosis, hypoglycaemia; ciliary damage and thick mucus → paroxysms; air rushing through a narrowed glottis → whoop; raised intrathoracic pressure → post-tussive vomiting, subconjunctival haemorrhage; infant airway → apnoea and cyanosis.
- Pneumocystis interstitial inflammation → severe hypoxaemia with few crackles; interstitial "ground glass" on X-ray.
- Hydrocarbon dissolving surfactant → chemical pneumonitis → cough, fast breathing after ingestion.
- Hypoxaemia, sepsis and hyponatraemia (SIADH) → convulsions, reduced consciousness.
- Malnutrition and HIV → blunted fever and tachypnoea, so signs underestimate severity.

5. Classification / Staging

WHO/IMCI classification of cough or difficult breathing (2–59 months; UCG 2023; WHO 2024 guideline)

Classification	Signs	Treatment
Severe pneumonia (or very severe disease)	Cough or difficult breathing + **any**: general danger sign; **central cyanosis or SpO_2 <90%**; **severe respiratory distress** (grunting, very severe chest indrawing, head nodding); (in UCG also: extrapulmonary features in malnourished or HIV-positive children)	**Admit or refer** after first dose of antibiotic; oxygen; **ampicillin (or benzylpenicillin) + gentamicin**
Pneumonia	**Fast breathing and/or chest indrawing**, no danger sign	**Oral amoxicillin 40 mg/kg twice daily for 5 days** (3 days for fast breathing alone in low-HIV areas); review in 2–3 days
No pneumonia: cough or cold	None of the above	Home care, soothing remedy, **no antibiotic**; cough >14 days → TB, asthma, pertussis

UGANDA CONTEXT

The **WHO 2024 guideline on pneumonia and diarrhoea in children up to 10 years** confirmed that **chest-indrawing pneumonia without danger signs** is treated with **oral amoxicillin for 5 days as an outpatient** (rather than injectable antibiotics in hospital), and allowed **3 or 5 days** for fast-breathing pneumonia (3 days only in **low-HIV** settings). Uganda's adult HIV prevalence is about 5%, so **5 days** remains standard (UCG 2023). Uganda's **integrated community case management (iCCM)** lets VHTs give amoxicillin dispersible tablets for fast-breathing pneumonia in children 2–59 months.

Young infants (<2 months): UCG 2023: **consider every infant under 2 months with pneumonia as severe** (fast breathing ≥60, severe chest indrawing, grunting, not feeding, convulsions, drowsiness, apnoea, SpO_2 <90%) → admit, ampicillin + gentamicin. (WHO's 2015 possible serious bacterial infection guidance allows outpatient oral amoxicillin for **fast breathing alone in infants 7–59 days** when referral is impossible; know it exists, but in hospital practice we admit.)

Pneumonia in children ≥5 years (UCG 2023)

- **Moderate**: fever, chest pain, cough, RR >30, no chest indrawing → oral amoxicillin.
- **Severe**: plus chest indrawing, pulse >120, temperature >39.5 °C, danger signs, SpO_2 <90% → admit, IV benzylpenicillin or ampicillin (± gentamicin) or ceftriaxone.

Bronchiolitis severity (UCG; NICE): severe if **age <3 months**, RR >60–70, difficulty feeding (<50–75% of usual), **SpO_2 <92%**, apnoea, grunting, cyanosis, exhaustion.

Pertussis stages: catarrhal (1–2 weeks, most infectious) → paroxysmal (1–6 weeks) → convalescent (weeks to months, "100-day cough").

6. Clinical Features

History

Each question has a reason.

- **Cough and its duration** → <14 days acute; **≥14 days**: TB, asthma, pertussis, foreign body, HIV lung disease. Also difficult or fast breathing, fever, grunting, chest pain.
- **Age** → <2 months = severe by definition (neonatal sepsis organisms); <2 years with coryza and wheeze = bronchiolitis.

- **Danger signs**: able to drink or breastfeed? vomiting everything? convulsions? sleepy? → any one = severe pneumonia.
- **Paroxysms, whoop, post-tussive vomiting, apnoea or cyanosis** → pertussis (macrolide, isolate, contacts).
- **Sudden choking** → foreign body; **ingestion** of kerosene, paraffin or other chemicals → hydrocarbon pneumonitis.
- **Recent measles, skin boils or pyomyositis** → Staph.
- **HIV**: mother's status, infant's PCR or antibody, cotrimoxazole, ART → PCP and TB risk; ART decisions. Recurrent pneumonias.
- **TB contact** → TB workup.
- **Growth and nutrition**: weight-for-height Z-score, MUAC, oedema, breastfeeding → SAM changes antibiotics and fluids.
- **Immunisation**: PCV ×3, pentavalent ×3 (Hib, pertussis), MR ×2; BCG scar → aetiology and catch-up.
- **Treatment so far**: antibiotics (which, how long?) → treatment-failure pathway; herbal medicines; "nebulisation" at a clinic.
- **Home**: cooking fuel, smokers, crowding, siblings ill → prevention advice.

Examination

Follow the order of Hutchison's Clinical Methods (24th edition), adapted for children: **look before you touch**, count the breaths and listen to the chest while the child is calm on the mother's lap, and leave distressing parts (ears, throat) to last. In an emergency the ETAT assessment (section 9) comes first.

Introduction, consent and preparation

- Introduce yourself; confirm the child's name, age and the carer's relationship; explain; consent from the parent and assent from the child.
- **Hand hygiene**.
- **Position and exposure**: on the mother's lap, sitting up; expose the chest fully (you cannot see indrawing through clothes) in a warm room.

General inspection (end of the bed)

Sign	Mechanism	What it means
Lethargic, unconscious, unable to drink	Hypoxaemia, sepsis	**Danger sign: severe pneumonia**
Central cyanosis (tongue, lips)	Hypoxaemia	Severe pneumonia; oxygen
Nasal flaring, grunting, **head nodding**	Increased work; keeping alveoli open	Severe respiratory distress
Lower chest wall indrawing	Stiff lungs on a compliant chest wall	Pneumonia (at least)
Paroxysms with whoop or apnoea	Pertussis	Admit infants
Kerosene smell	Hydrocarbon ingestion	Do not induce vomiting
Wasting, oedema	Malnutrition	SAM protocol; blunted signs
Measles rash, skin boils	Predisposition to Staph	Cloxacillin cover

Vital signs and growth: **count the RR for 60 seconds** while the child is calm (on the mother's lap, not feeding, not crying; watch the abdomen in infants); **SpO_2** with an appropriately sized probe (finger or toe in infants), good waveform; temperature, HR, capillary refill, hydration; **weight, length or height, WHZ, MUAC, oedema**.

Hands, face and neck

- **Hands**: clubbing (bronchiectasis, chronic HIV lung disease, TB); pallor; cold peripheries and CRT >3 s (shock, uncommon in pneumonia).

- **Face and mouth**: central cyanosis; oral thrush and parotid swelling (HIV); subconjunctival haemorrhage (pertussis); measles rash, Koplik spots.
- **Neck**: lymphadenopathy (HIV, TB); trachea position (shift away from an effusion or tension pneumothorax, towards collapse); neck stiffness (meningitis).

Respiratory system

Step	Sign	What it means
Inspection	Fast breathing for age; lower chest wall indrawing; asymmetry	Pneumonia; severity; effusion or pneumothorax
Palpation	Trachea deviated; reduced expansion on one side	Effusion, empyema, pneumothorax, collapse
Percussion	Dull	Consolidation
Percussion	**Stony dull**	Effusion or empyema
Percussion	Hyper-resonant one side	Pneumothorax
Auscultation	Bronchial breathing, focal crackles	Consolidation
Auscultation	Reduced or absent breath sounds	Effusion, empyema, collapse, foreign body
Auscultation	Fine crackles and wheeze under 2 years	Bronchiolitis
Auscultation	Wheeze	Viral wheeze or asthma: re-count after a bronchodilator
Auscultation	Clear chest between paroxysms	Pertussis

Completing the examination

- **Heart**: murmur, gallop (heart failure: VSD, RHD, myocarditis; severe anaemia).
- **Abdomen**: liver size (heart failure), hepatosplenomegaly (HIV).
- **Skin**: boils, pyomyositis, measles rash; **BCG scar**; signs of SAM.
- **Fontanelle and neck stiffness**: meningitis.
- **Ears and throat, last**: otitis media.
- **Thank**, **wash hands**, **summarise** with the WHO classification and SpO_2.

Document these positives	And these negatives
RR (counted for 60 s) and age cut-off	"No general danger signs"
Chest indrawing; grunting; head nodding	"No central cyanosis"
SpO_2 in air (and on oxygen)	"No wheeze" (or wheeze: re-count after bronchodilator)
Focal signs (dullness, bronchial breathing, crackles)	"No stony dullness; trachea central"
WHZ, MUAC, oedema, HIV status, vaccines	"No hepatomegaly or murmur"

Red flags

RED FLAG

Severe pneumonia, needs admission now: any general danger sign (unable to drink or breastfeed, vomits everything, convulsions, lethargy or unconsciousness), **SpO_2 <90%** or central cyanosis, grunting, head nodding, very severe chest indrawing, RR ≥70, age <2 months, severe acute malnutrition, apnoea.

RED FLAG

Pneumonia that is not responding: still febrile and breathless after 48 h on correct antibiotics, rising RR, new dullness (empyema), sudden worsening (pneumothorax), pneumatocoeles (Staph), very low SpO_2 out of proportion to signs in an HIV-exposed infant (PCP).

Thinking like a clinician

- **Count, don't guess.** "Fast breathing" by impression is wrong half the time. Use the respiratory timer and a full minute.
- **Fever + fast breathing in Uganda = pneumonia and/or malaria.** Do an RDT on every febrile child; **treat both** if both are present. Severe malaria with acidosis also causes deep, fast breathing.
- **Wheeze changes the story.** Fast breathing with wheeze in an infant → bronchiolitis or viral wheeze; in an older child → asthma (give salbutamol and **re-count**: if fast breathing resolves, it was bronchospasm, not pneumonia). WHO advises a rapid-acting bronchodilator trial before classifying wheezing children with fast breathing.
- **The thin, HIV-exposed infant with severe hypoxaemia and few chest signs** has PCP until proven otherwise: high-dose cotrimoxazole + steroid, alongside standard antibiotics.
- **Staph clues**: measles, skin boils or pyomyositis, rapid deterioration, pneumatocoeles, empyema, pneumothorax.
- **Persistent fever and cough >2 weeks with weight loss** → TB (see the TB topic); **sudden onset in a toddler** → foreign body.
- **Basal crackles + hepatomegaly + murmur** → heart failure (VSD, RHD, myocarditis): fluids and diuretics, not just antibiotics.
- **SAM + pneumonia**: signs are blunted; treat as complicated SAM (antibiotics for all, careful fluids, F-75, warmth, glucose).
- **A child with danger signs is severe whatever the SpO_2.**

7. Differential Diagnoses

Differential	Distinguishing clinical feature	Distinguishing investigation
Pneumonia	Fever, cough, fast breathing, indrawing, focal crackles or bronchial breathing	Clinical; SpO_2; CXR if severe
Bronchiolitis	<2 y, coryza then cough, fine crackles and wheeze, hyperinflation, little fever	Clinical
Asthma / viral wheeze	Recurrent wheeze, responds to bronchodilator, atopy	Bronchodilator trial
Malaria (severe, with acidosis)	Deep (acidotic) breathing, fever, pallor, prostration, clear chest	RDT or blood smear, lactate
Heart failure (VSD, PDA, RHD, cardiomyopathy, severe anaemia)	Hepatomegaly, gallop, murmur, poor feeding or sweating, oedema	CXR (cardiomegaly), Hb, echo
Pulmonary TB	Cough ≥2 weeks, weight loss, TB contact, non-response to antibiotics	Xpert, CXR, TST
PCP (HIV)	HIV-exposed or infected infant 2–6 months, severe hypoxaemia, few crackles, high LDH	HIV PCR or test; CXR interstitial
Foreign body aspiration	Sudden choking, unilateral signs	Inspiratory and expiratory CXR; bronchoscopy

Differential	Distinguishing clinical feature	Distinguishing investigation
Pertussis	Paroxysms, whoop, vomiting, apnoea; clear chest between bouts; lymphocytosis	Clinical; PCR (rare)
Hydrocarbon aspiration	Kerosene smell, cough after ingestion	History; CXR at 6 h
Metabolic acidosis (DKA, severe dehydration, salicylate)	Deep sighing breathing, clear chest	Glucose, ketones, gas
Sepsis / meningitis	Fast breathing from fever and acidosis; neck stiffness, bulging fontanelle	LP, blood culture

8. Clinical Workup

"Available at" is the lowest level that usually offers the test Verify *compiled from the source notes, not an MoH list*.

Bedside

Test	Why / how	Expected finding	How it changes management	Available at
Respiratory rate (timer)	60 s, calm child	Fast breathing by age	Classifies pneumonia	All levels, including VHT
Pulse oximetry	Right-sized probe; warm extremity; steady trace for 30–60 s	**<90% = severe pneumonia, give oxygen**; 90–93% = monitor closely	Oxygen, admission	HC IV and above; improving at HC III; some VHTs
Malaria RDT	Every febrile child	Positive	Co-treat malaria	All levels
Random blood glucose	Lethargic, not feeding, SAM	Hypoglycaemia	10% dextrose 5 mL/kg	HC IV and above; variable at HC III
Haemoglobin (HemoCue)	Pallor, severe illness	Severe anaemia; heart failure from anaemia mimics pneumonia	Transfuse if Hb ≤4 g/dL, or 4–6 g/dL with respiratory distress, shock or impaired consciousness	HC III/IV and above Verify
HIV rapid test / DNA PCR	Status unknown; infants of positive mothers need PCR	Positive	PCP risk; ART; cotrimoxazole	Rapid test all levels; PCR sample referred (hub system)
MUAC tape	6–59 months	<11.5 cm = SAM	SAM protocol	All levels
Bronchodilator trial	Wheeze + fast breathing: salbutamol 2 puffs via spacer or 2.5 mg nebulised, re-count after 15–20 min	Fast breathing resolves	Bronchospasm, not pneumonia	All levels with salbutamol

Basic lab

Test	When	Expected / abnormal finding	How it changes management	Available at
Full blood count	Severe or complicated	Neutrophilia (bacterial); lymphocytosis >20 × 10^9/L (pertussis); anaemia; low platelets	Supports diagnosis; transfusion	HC IV and above
Urea, electrolytes	Severe illness, IV fluids, SAM	Hyponatraemia (SIADH), AKI	Fluid restriction (about 2/3 maintenance if Na low)	General hospital and above Verify
GeneXpert (sputum, gastric aspirate, **stool**, NPA)	Cough ≥2 weeks, TB contact, HIV, not responding	MTB detected ± rifampicin resistance	TB treatment	HC IV and hospitals; samples referred from lower levels

Imaging

Test	When	Expected finding	How it changes management	Available at
Chest X-ray	Severe pneumonia, not responding at 48 h, suspected complications (effusion, empyema, pneumothorax, pneumatocoeles), suspected TB or foreign body. **Not needed** for non-severe pneumonia	Lobar consolidation (pneumococcus); patchy bronchopneumonia; **pneumatocoeles, empyema** (Staph); hilar nodes, miliary pattern (TB); diffuse interstitial "ground glass" (PCP); hyperinflation (bronchiolitis)	Staph → cloxacillin; empyema → drain; TB → Xpert, treat; PCP → cotrimoxazole	HC IV (some), all hospitals
Chest ultrasound	Suspected effusion or empyema	Fluid, septations, size	Guides tap and drain site	RRH, NRH, some HC IVs

Specialised / referral-level

Test	When	Expected finding	How it changes management	Available at
Blood culture	Severe pneumonia, before antibiotics if possible	Pneumococcus, Staph, *Klebsiella*, *Salmonella*	Tailors therapy; yield low	RRH, NRH only
Pleural fluid (diagnostic tap)	Effusion	Pus; pH <7.2, glucose low, LDH high (empyema); lymphocytes, high protein (TB); Gram stain, culture, **Xpert**	Chest drain for empyema; TB treatment	Hospitals with ultrasound
Nasopharyngeal swab PCR (RSV, pertussis, influenza)	Research and outbreaks, private hospitals	Viral or pertussis diagnosis	Isolation, macrolide for pertussis	Rarely available
Blood gas	Severe distress, CPAP, HDU	Rising pCO_2	CPAP or ventilation	NRH, ICU
Bronchoscopy	Suspected foreign body	Object seen and removed	Definitive	NRH (Mulago ENT/surgery)

Putting it together

Model one-line summary

> "A 10-month-old girl weighing 7.2 kg (WHZ −1.5, MUAC 12.4 cm), fully immunised for age including PCV ×3, HIV-exposed with a negative DNA PCR at 6 weeks and still breastfeeding (repeat test due), with 3 days of cough and fever, now unable to breastfeed, RR 64, lower chest wall indrawing, grunting and SpO_2 86% in air, malaria RDT negative: **severe pneumonia with hypoxaemia**."

Problem list

1. Severe pneumonia (danger sign + hypoxaemia).
2. Hypoxaemia: oxygen.
3. HIV-exposed infant, breastfeeding: repeat HIV test; consider PCP if hypoxaemia severe and test positive.
4. Moderate wasting risk (WHZ −1.5): monitor feeding.

Working diagnosis: *severe community-acquired pneumonia (likely pneumococcal) with hypoxaemia in an HIV-exposed infant.*

Model case presentation (ward round)

> "Namubiru is a 10-month-old girl from Nansana, weighing 7.2 kg (WHZ −1.5, MUAC 12.4 cm, no oedema), up to date with UNEPI vaccines for age including PCV and pentavalent ×3 and MR1 at 9 months, HIV-exposed with a negative DNA PCR at 6 weeks, still breastfeeding plus family foods. She presented with 3 days of cough and fever and was unable to breastfeed today. On arrival: lethargic but rousable, T 39.0 °C, RR 64 (counted for 60 s), HR 168, SpO_2 86% in air, lower chest wall indrawing, grunting, bronchial breathing in the right upper zone; no wheeze, no hepatomegaly; malaria RDT negative; RBS 4.8 mmol/L. Classification: **severe pneumonia with hypoxaemia**. She is on oxygen 1 L/min via nasal prongs (SpO_2 95%), IV ampicillin 360 mg 6-hourly and gentamicin 54 mg once daily, NG expressed breast milk, paracetamol. Plan: monitor RR, SpO_2 3-hourly; repeat HIV test (rapid antibody at 9 months+, PCR if positive); review at 48 h for failure; CXR if not improving; nutritional follow-up."

EXAM FAVOURITE

Worked example 1: classify three children with cough. (a) 8 months, RR 56, no indrawing, feeding well, SpO_2 96% → fast breathing (≥50) → **pneumonia** → amoxicillin 40 mg/kg twice daily × 5 days at home, review in 2–3 days. (b) 3 years, RR 38, lower chest wall indrawing, drinking, SpO_2 94% → indrawing → **pneumonia** (WHO 2014/2024: outpatient oral amoxicillin 5 days, unless other risks such as SAM or HIV infection; many Ugandan clinicians still admit). (c) 18 months, RR 52, not able to drink, SpO_2 88% → **severe pneumonia** → admit, oxygen, ampicillin + gentamicin.

EXAM FAVOURITE

Worked example 2: the 48-hour review. A 2-year-old (11 kg) on ampicillin + gentamicin for 48 h is still febrile (39.5 °C), RR 60, SpO_2 89% on 1 L/min, with new stony dullness and absent breath sounds over the right base. CXR: right pleural effusion with a pneumatocoele in the left upper zone. **Interpretation**: treatment failure from **staphylococcal pneumonia with empyema**. Action: ultrasound-guided **diagnostic tap** (thick pus) → **chest drain**; switch to **cloxacillin 50 mg/kg (550 mg) 6-hourly + gentamicin 7.5 mg/kg once daily** for at least 3 weeks; increase oxygen; blood culture; HIV test; nutritional support.

9. Management

Immediate / emergency

First 5 minutes

1. **ETAT triage**: Airway and Breathing emergency signs (obstructed breathing, **central cyanosis, severe respiratory distress**), circulation (cold hands + CRT >3 s + weak fast pulse), coma or convulsions, severe dehydration. If present → resuscitation area.
2. **Oxygen** for SpO_2 <90% or any emergency sign (target **≥94% during resuscitation**, then ≥90%).
3. **Position**: sitting up on the mother's lap; clear the nose with gentle suction.
4. **Check glucose** (lethargic, convulsing or not feeding): treat hypoglycaemia <2.5 mmol/L (or <3 mmol/L in SAM) with **10% dextrose 5 mL/kg IV** (or 50 mL of 10% dextrose or sugar water by NG if no IV).
5. **Convulsions**: diazepam 0.5 mg/kg rectally or 0.2–0.3 mg/kg IV (UCG 2023); look for meningitis and malaria.
6. **Weigh** the child; **first dose of antibiotics** without delay in severe pneumonia (before referral if you are at a lower level).
7. **Shock** is uncommon in pneumonia; if present (and no SAM), give 10–20 mL/kg Ringer's lactate or normal saline and reassess; think sepsis and dehydration.

ABCDE summary: **A** clear the nose, sit up; **B** SpO_2, oxygen, RR; **C** CRT, pulse, bolus only if shocked and not SAM; **D** AVPU, glucose, convulsions; **E** temperature, rash, skin sepsis, weight, MUAC.

Choking (foreign body): effective cough → encourage coughing. Ineffective cough: infant – 5 back blows + 5 chest thrusts; child >1 y – 5 back blows + 5 abdominal thrusts; unconscious → CPR.

Definitive

A. Pneumonia (non-severe): outpatient

1. **Oral amoxicillin dispersible tablets (250 mg) 40 mg/kg per dose twice daily for 5 days** (UCG 2023; WHO 2024 minimum 80 mg/kg/day):
   - 2–12 months (4–<10 kg): **250 mg (1 tab)** twice daily
   - 1–3 years (10–<14 kg): **500 mg (2 tabs)** twice daily
   - 3–5 years (14–19 kg): **750 mg (3 tabs)** twice daily
2. **Wheeze**: salbutamol inhaler with spacer 1–2 puffs (or more) 4–6-hourly until wheeze stops (UCG 2023).
3. **Fever**: paracetamol 15 mg/kg 6-hourly.
4. **Home care**: continue breastfeeding and feeding, extra fluids, clear the nose, keep warm (not overwrapped).
5. **Return immediately** if unable to drink or breastfeed, breathing faster or harder, becomes sicker, convulsions.
6. **Review after 2–3 days** (IMCI/UCG): better → complete course; same or worse → severe pathway (refer, injectable antibiotics, oxygen).

B. Severe pneumonia: inpatient (UCG 2023, WHO Pocket Book 2013)

1. **Admit**; refer from HC III/IV after the first dose of antibiotics (IM if no IV access).
2. **Oxygen** if SpO_2 <90% (section E).
3. **Antibiotics (first line)**: **ampicillin 50 mg/kg IV/IM 6-hourly** (or **benzylpenicillin 50,000 IU/kg IV/IM 6-hourly**) **plus gentamicin 7.5 mg/kg IV/IM once daily**, for **at least 5 days** (up to 10). Switch to oral amoxicillin 40 mg/kg twice daily when improving, to complete at least 5 days in total (UCG) (WHO: total 5–10 days).
4. **Second line** (no improvement after **48 h**, or very severe): **ceftriaxone 80 mg/kg IV/IM once daily** (UCG 2023; max 2 g), 7–10 days.
5. **Suspected Staph** (pneumatocoeles, empyema, skin sepsis, after measles, rapid deterioration): **cloxacillin 50 mg/kg IV/IM 6-hourly + gentamicin 7.5 mg/kg once daily**, for **at least 3 weeks** (UCG: 21 days).

6. **Last resort** if the above are unavailable: chloramphenicol 25 mg/kg IV 6-hourly (not in neonates <7 days or preterm).
7. **Supportive care**:
   - **Fluids**: encourage breastfeeding and oral fluids; NG feeds or IV fluids if unable to drink. Give **maintenance only** (Holliday–Segar), and consider restricting to about two-thirds of maintenance if very severe or hyponatraemic (SIADH). Avoid overload, especially in small and malnourished children (UCG).
   - **Fever**: paracetamol 15 mg/kg 6-hourly (UCG 10 mg/kg 4–6-hourly).
   - **Wheeze**: salbutamol 1–2 puffs (or more via spacer) 4–6-hourly.
   - **Gentle suction** of thick upper-airway secretions; **no chest physiotherapy** in acute pneumonia.
   - **Convulsions**: diazepam 0.5 mg/kg rectally or 0.2–0.3 mg/kg IV; if continuous, phenobarbital 15–20 mg/kg IV/IM loading (UCG lists 10–15 mg/kg IM), then 3–5 mg/kg/day.
   - **Glucose**: prevent hypoglycaemia (feeds; dextrose if not feeding).
8. **Treat co-existing conditions**: malaria (IV artesunate if severe), SAM (WHO 2023 SAM protocol), anaemia (transfuse if Hb ≤4 g/dL, or 4–6 g/dL with respiratory distress, shock or impaired consciousness), HIV (cotrimoxazole, ART).

C. Young infants (<2 months)

- **Admit all** (UCG 2023). Keep warm (skin-to-skin), breastfeed or expressed milk by NGT, prevent hypoglycaemia.
- **Ampicillin 50 mg/kg IV** (WHO: **12-hourly in week 1, 8-hourly in weeks 2–4**, 6-hourly from 1 month; UCG 2023 uses 6-hourly at all ages) **+ gentamicin 5 mg/kg once daily** (term, first week of life) / **7.5 mg/kg once daily** (>7 days); LBW in week 1: **3 mg/kg** (WHO; UCG 4 mg/kg preterm).
- Severely ill: **ceftriaxone 100 mg/kg once daily** (UCG 2023) (avoid with calcium-containing IV fluids and in jaundiced neonates; cefotaxime preferred where available).
- Oxygen to keep SpO_2 **≥90%** (UCG: >94% for neonates with emergency signs).
- At least 5 days, and 3 days after the infant is well; **10 days** if septicaemia; **21 days** if meningitis (UCG).

D. Hypoxaemia and oxygen therapy (WHO 2016 oxygen therapy for children; UCG 2023)

Who gets oxygen

- **SpO_2 <90%** (at Ugandan altitudes; <87% above 2,500 m) → oxygen.
- **Emergency signs** (obstructed or absent breathing, severe respiratory distress, central cyanosis, shock, coma, convulsions) → oxygen during resuscitation to **SpO_2 ≥94%**.
- **No oximeter**: give oxygen if **central cyanosis, grunting, nasal flaring, head nodding, inability to drink or feed because of breathing difficulty, severe lower chest wall indrawing, RR ≥70, or depressed consciousness** (WHO 2016; WHO 2024 adds head nodding, nasal flaring, grunting or RR ≥20 above the age cut-off as signs of hypoxaemia).

Delivery

Device	Flow	FiO_2 approx	Notes
Nasal prongs (preferred)	**0.5–1 L/min neonates; 1–2 L/min infants; 2–4 L/min older children**	30–35% (at 1–2 L/min in infants)	Comfortable; child can feed; no humidification needed at these flows
Nasal catheter (8 Fr, inserted to the distance nose–inner eyebrow)	0.5–2 L/min	35–40%	Needs humidification if high flow; risk of gastric distension
Face mask	5–10 L/min (≥5 to avoid CO_2 rebreathing)	40–60%	Older children, emergencies

Device	Flow	FiO_2 approx	Notes
Head box	2–3 L/min/kg	Variable	Neonates; wasteful, CO_2 build-up
Bubble CPAP	5–10 cm H_2O via nasal prongs	–	Severe pneumonia or bronchiolitis not improving on standard oxygen (RRH, NRH)

Weaning

- **Trial off oxygen daily** when stable: if SpO_2 stays **≥90% in room air for 10–15 minutes** (and the child is not distressed), stop and recheck after 1 h and then 3-hourly for the first day.
- Concentrators need constant power: keep a cylinder as back-up; use one concentrator with a flow splitter to treat several children only if each child's flow is measured.

> UGANDA CONTEXT
> Uganda's 2023 guideline (UCG) says oxygen when **SpO_2 <90%** for children with pneumonia, and **>94%** for young infants with emergency signs. NICE uses 92% (bronchiolitis). For exams in Uganda: **<90% = give oxygen**.

E. Bronchiolitis (UCG 2023 5.2.1; NICE NG9)

1. **Diagnose**: <2 years (usually <1), coryza 1–3 days, then cough, fast breathing or recession, and wheeze and/or crackles. Worst on days 3–5.
2. **Assess severity** (section 5).
3. **Mild–moderate** (feeding, no hypoxaemia): home care, saline drops, small frequent feeds, paracetamol for fever, return if worse.
4. **Admit if severe**:
   - **Oxygen** to keep SpO_2 **≥90%** (WHO) / ≥92% (NICE): nasal prongs 1–2 L/min (UCG: humidified 1–2 L/min).
   - **Feeding**: small frequent breastfeeds; **NG or OG feeds** if not feeding adequately; IV fluids only if NG not tolerated (isotonic, about maintenance).
   - **Gentle nasal suction** if secretions block the nose.
   - **CPAP** if impending respiratory failure.
5. **What not to give** (NICE NG9): antibiotics (unless secondary bacterial infection), salbutamol, adrenaline, hypertonic saline, steroids, ipratropium, montelukast, chest physiotherapy.

> UGANDA CONTEXT
> **UCG 2023 differs from NICE**: for severe bronchiolitis UCG suggests a **trial of salbutamol** (2 puffs via spacer every 30 min or 2.5 mg nebulised; **continue 6-hourly only if it helps, stop if no response**) and **nebulised adrenaline 1:1000, 1 mL in 2–4 mL saline, every 2–4 h**; it also lists total fluids of **150 mL/kg/24 h** plus losses. NICE NG9 and WHO recommend **no bronchodilators** and **no more than maintenance fluids** (overload and SIADH risk). On Kampala wards most consultants follow the supportive (NICE/WHO) approach and allow a single salbutamol trial in older infants with wheeze Verify *current ward practice*. Check your unit's practice.

Discharge: SpO_2 ≥90–92% in air for 4 h (including sleep), feeding ≥75% of normal, clinically stable; counsel that cough may last 3 weeks.

F. Pertussis (UCG 2023 5.2.8)

1. **Recognise**: catarrhal stage (1–2 weeks, most infectious), **paroxysmal stage** (1–6 weeks: bouts of coughing ending in a **whoop**, **post-tussive vomiting**, subconjunctival haemorrhages; infants <6 months: **apnoea and cyanosis**, may not whoop), convalescent stage (weeks to months, "100-day cough").

2. **Admit** infants <6 months, any with apnoea, cyanosis, pneumonia, convulsions, dehydration or malnutrition.
3. **Antibiotics** (within 3 weeks of cough onset, UCG): **azithromycin** (preferred, especially <1 month) or **erythromycin 12.5 mg/kg 6-hourly** (UCG 10–15 mg/kg) for 7 days (UCG; 14 days classically). Reduces infectiousness; changes the course only if early.
4. **Supportive**: oxygen for cyanosis or apnoea (SpO_2 <90%), **gentle suction** of secretions after paroxysms, **small frequent feeds** (re-feed after vomiting), NG feeds if needed, monitor for apnoea (put near the nursing station).
5. **Do NOT give**: cough mixtures, sedatives, mucolytics, antihistamines (UCG: "useless in pertussis").
6. **Isolate** until 5 days of antibiotics.
7. **Contacts**: macrolide prophylaxis for household contacts (especially infants and pregnant women); complete DTP vaccination; DTP at 6, 10, 14 weeks (UNEPI). **Notifiable.**

G. Empyema and parapneumonic effusion

1. Suspect: **persistent fever** after 48–72 h of antibiotics, **stony dull percussion**, reduced or absent breath sounds, mediastinal shift.
2. **CXR + ultrasound**; **diagnostic tap** (pus, Gram stain, culture, **Xpert**, pH, glucose).
3. **Chest drain** (intercostal, 5th intercostal space mid-axillary line, underwater seal) for frank pus or large or complicated effusion; surgery (decortication) at NRH for loculated empyema not draining.
4. **Antibiotics**: **cloxacillin 50 mg/kg 6-hourly + gentamicin 7.5 mg/kg once daily** (Staph cover), or ceftriaxone if pneumococcal; continue **3–4 weeks** (IV until afebrile, then oral).
5. **Exclude TB** (lymphocytic exudate, TB contact, HIV).

H. HIV-related pneumonia

- **HIV-exposed or infected infant with severe pneumonia**: standard antibiotics **plus high-dose cotrimoxazole for PCP** (especially age 2–6 months, severe hypoxaemia, few chest signs) for **21 days**, **prednisolone** if hypoxaemic, then cotrimoxazole prophylaxis. Test (DNA PCR <18 months), and start **ART** within 2 weeks (not delayed by PCP; TB requires attention to timing).
- Consider **TB**, CMV, LIP, bronchiectasis in HIV-positive children with chronic or recurrent pneumonia.

I. Aspiration

Situation	Management
Kerosene, paraffin or hydrocarbon ingestion	**Do not induce vomiting, no gastric lavage, no activated charcoal**. Remove contaminated clothes, wash skin. Oxygen if SpO_2 <90%. **Observe ≥6 hours**; CXR at 6 h if symptomatic. Antibiotics only if secondary bacterial pneumonia (fever after 24–48 h with new signs).
Foreign body (groundnut, bean, maize, bead)	Acute choking: first aid (Immediate / emergency). Later presentation (unilateral wheeze, recurrent pneumonia in one lobe) → inspiratory and expiratory CXR, **rigid bronchoscopy** (Mulago ENT/surgery).
Aspiration pneumonia (cerebral palsy, reduced consciousness, swallowing problems)	Ampicillin + gentamicin + **metronidazole**, or amoxicillin-clavulanate; positioning, feeding assessment, NG feeding.

Drug table

Drug	Dose (per kg + max for children; adult)	Route	Frequency	Duration	Key cautions
Amoxicillin DT (non-severe pneumonia; step-down)	**40 mg/kg**, max 1 g/dose (by age: 1, 2 or 3 × 250 mg tabs); adult 500 mg–1 g 8-hourly Verify	Oral	**12-hourly**	**5 days** (3 days fast breathing only in low-HIV settings)	Dissolve in breast milk or water
Ampicillin (severe pneumonia, infants)	**50 mg/kg**, max 2 g/dose	IV/IM	**6-hourly** (12-hourly week 1, 8-hourly weeks 2–4, WHO)	≥5 days	Switch to oral amoxicillin when improving
Benzylpenicillin (severe pneumonia, alternative)	**50,000 IU/kg**, max 2–4 MU/dose	IV/IM	6-hourly	≥5 days	–
Gentamicin (with ampicillin)	**7.5 mg/kg** (>1 week old); **5 mg/kg** (first week); LBW week 1 **3 mg/kg** (WHO; UCG 4 mg/kg preterm)	IV/IM	**Once daily**	≥5 days	Renal and ototoxicity; avoid courses >7–10 days without levels
Ceftriaxone (second line; severely ill infants)	**80 mg/kg** (100 mg/kg in severely ill infants, UCG), max 2 g (4 g meningitis)	IV/IM	Once daily	7–10 days	Not with IV calcium in neonates; avoid in jaundiced neonates (cefotaxime preferred); AWaRe Watch
Cloxacillin (Staph pneumonia, empyema)	**50 mg/kg** (25–50), max 2 g/dose	IV/IM then oral	**6-hourly**	**≥21 days** (3–4 weeks empyema)	With gentamicin
Chloramphenicol (if others unavailable)	**25 mg/kg**	IV	6-hourly	5–10 days	Not <7 days old or preterm (grey baby syndrome)
Cotrimoxazole (high dose) (PCP)	**120 mg/kg/day** of combined drug (= TMP 20 mg/kg/day) in 3–4 divided doses (UCG); WHO: TMP 8 mg/kg + SMX 40 mg/kg 8-hourly	Oral/IV	6–8-hourly	**21 days**, then prophylaxis	Rash, marrow suppression
Prednisolone (PCP with hypoxaemia)	**2 mg/kg/day** in divided doses for 5 days then taper over 21 days (UCG; WHO 1–2 mg/kg/day for 1 week)	Oral	Divided doses	About 21 days total	–
Azithromycin (pertussis)	**<6 months: 10 mg/kg once daily × 5 days; ≥6 months: 10 mg/kg day 1 (max 500 mg), then 5 mg/kg days 2–5 (max 250 mg)**	Oral	Once daily	5 days	Preferred in neonates (CDC); fewer GI effects
Erythromycin (pertussis, UCG first line; *Mycoplasma*)	**12.5 mg/kg** (UCG 10–15 mg/kg)	Oral	**6-hourly**	**7 days** (UCG) to 14 days	Avoid in <1 month (pyloric stenosis)
Doxycycline (*Mycoplasma* in **>8 y**)	2 mg/kg, max 100 mg	Oral	12-hourly	7–10 days	Not <8 y

Drug	Dose (per kg + max for children; adult)	Route	Frequency	Duration	Key cautions
Metronidazole (aspiration pneumonia, lung abscess)	7.5 mg/kg (UCG 12.5 mg/kg IV for lung abscess)	IV/oral	8-hourly	With penicillin; lung abscess 4–6 weeks	–
Paracetamol (fever, pain)	15 mg/kg (UCG 10 mg/kg 4–6-hourly), max 60 mg/kg/day; adult 1 g	Oral	6-hourly	As needed	–
Salbutamol (wheeze)	100 µg/puff, 2 puffs (or more) via spacer; or 2.5 mg nebulised	Inhaled	4–6-hourly	Until wheeze stops	Re-count RR after a trial
Diazepam (convulsions)	0.5 mg/kg rectally or 0.2–0.3 mg/kg IV	PR/IV	Once, repeat once if needed Verify	–	Respiratory depression; look for meningitis, malaria
Phenobarbital (continuing convulsions)	Load 15–20 mg/kg (UCG 10–15 mg/kg IM), then 3–5 mg/kg/day	IV/IM	Load, then daily	–	Respiratory depression
10% dextrose (hypoglycaemia)	5 mL/kg	IV (or 50 mL by NG)	Once, recheck	–	Hypoglycaemia <2.5 mmol/L (<3 in SAM)
Furosemide (heart failure)	1 mg/kg	IV/oral	Verify *frequency*	–	Hypokalaemia

DRUG DOSING

Gentamicin once daily is standard: 7.5 mg/kg for children over 1 week old (UCG 2023), 5 mg/kg in the first week of life. Write the actual mg dose using today's weight, and avoid courses longer than 7–10 days without levels.

Referral criteria

Setting	What to do	Refer up when
VHT (iCCM)	Count RR with a timer; fast breathing (2–59 months) → amoxicillin DT 5 days	Any danger sign or chest indrawing → first dose amoxicillin and refer
HC III	Classify; pneumonia → amoxicillin 5 days	Severe → first dose IM ampicillin or benzylpenicillin + gentamicin, oxygen if available, refer with a referral note
HC IV / General Hospital	Admit severe pneumonia; oxygen; IV/IM ampicillin + gentamicin; ceftriaxone if failing; CXR if available	Empyema needing a drain; CPAP; failure despite second line → RRH
RRH	All above, chest drains, CPAP, blood cultures, Xpert, HDU	Ventilation, bronchoscopy, decortication → NRH
NRH (Mulago)	Bronchoscopy, surgery, PICU	n/a

Referral checklist: first antibiotic dose given; oxygen during transfer; glucose checked; keep warm and breastfeeding; referral note with times and doses.

Monitoring and follow-up

- **Inpatients** (UCG 2023): RR every 2 hours (more often if very sick), temperature 6-hourly, **SpO_2 at least every 3–6 hours** (UCG minimum 12-hourly; every 3 h while on oxygen is better practice), feeding, alertness, work of breathing.
- **On oxygen**: SpO_2 at least every **3 hours**; check prongs are in the nose, tubing unkinked, concentrator working.
- **Review at 48 hours** for treatment failure (mnemonic "STEP-HIV-F", section 14).
- **Outpatient pneumonia**: review at **2–3 days**.

Discharge and prevention of recurrence

- Discharge when off oxygen ≥24 h, feeding well, afebrile, improving; complete antibiotics at home.
- **Before discharge**: check and **catch up vaccines** (PCV, pentavalent, MR), HIV status, nutrition (enrol in OTC or ITC if SAM or MAM), counsel on smoke, breastfeeding, handwashing, danger signs; follow up in 1–2 weeks.

10. Complications

Disease-related

Complication	Clues	Action
Hypoxaemia / respiratory failure	SpO_2 <90%, exhaustion	Oxygen, CPAP, ICU
Parapneumonic effusion / empyema	Persistent fever, stony dullness	Tap, drain, prolonged antibiotics
Lung abscess / pneumatocoeles	Staph, cavities	Cloxacillin; most pneumatocoeles resolve
Pneumothorax / pyopneumothorax	Sudden deterioration, hyper-resonance	Needle decompression, chest drain
Sepsis, meningitis	Shock, neck stiffness, bulging fontanelle	LP, meningitic doses
Hyponatraemia (SIADH)	Seizures, lethargy	Fluid restriction, sodium correction
Heart failure	Hepatomegaly, gallop	Fluid restriction, furosemide 1 mg/kg
Malnutrition, growth faltering	Weight loss	Nutritional rehabilitation
Pertussis complications	New fever, apnoea, fits	Pneumonia, apnoea, convulsions, encephalopathy, malnutrition, dehydration, hernias, rectal prolapse, subconjunctival haemorrhage
Long-term	Bronchiectasis (after measles, pertussis, recurrent pneumonia), post-infectious bronchiolitis obliterans	Physiotherapy, specialist

Treatment-related

Complication	Cause	Recognition and action
Fluid overload, hyponatraemia	Excess IV fluids (and UCG's 150 mL/kg/day in bronchiolitis)	Maintenance or two-thirds maintenance; weigh, check sodium
Nephrotoxicity, ototoxicity	Gentamicin, prolonged courses	Once daily, ≤7–10 days without levels
Calcium precipitation; kernicterus risk	Ceftriaxone in neonates	No IV calcium; avoid in jaundiced neonates
Grey baby syndrome	Chloramphenicol in neonates	Not under 7 days or preterm
Pyloric stenosis	Erythromycin under 1 month	Use azithromycin
Rash, marrow suppression	High-dose cotrimoxazole	FBC; stop if severe rash
Gastric distension, nasal trauma	Nasal catheter oxygen	Prefer prongs
Aspiration	Inducing vomiting after kerosene	Never induce vomiting
Respiratory depression	Sedatives, cough mixtures in pertussis	Do not give

11. Prognosis

- **Hypoxaemia** is the strongest predictor of death: a child with SpO_2 <90% is several times more likely to die.
- Most children with non-severe pneumonia recover on 5 days of oral amoxicillin.
- **Bronchiolitis** peaks on days 3–5; cough resolves within 3 weeks in 90% (NICE NG9).
- **Pertussis**: the convalescent stage lasts weeks to months ("100-day cough"); young infants are at greatest risk of apnoea and death.
- **Pneumatocoeles**: most resolve.
- **Long-term**: bronchiectasis after measles, pertussis or recurrent pneumonia; post-infectious bronchiolitis obliterans.
- Malnutrition and HIV blunt the signs and worsen the outcome.

12. Prevention

- **Exclusive breastfeeding** to 6 months.
- **Vaccines (UNEPI)**: **PCV and pentavalent (DTP-HepB-Hib) at 6, 10, 14 weeks; measles-rubella at 9 and 18 months**.
- **HIV**: cotrimoxazole prophylaxis for HIV-exposed infants; prevention of vertical transmission; early ART.
- **Nutrition and zinc**; treat malnutrition.
- **Clean air**: improved cookstoves, cook outside, no smoking indoors.
- **Handwashing**; reduce crowding.
- **Pertussis contacts**: macrolide prophylaxis (especially infants and pregnant women), complete DTP.
- **Safety**: store kerosene and paraffin out of reach and never in drink bottles Verify *general safety advice, not in the source guidelines*.

13. Special Populations

- **Young infants <2 months**: all pneumonia is severe; ampicillin dosing interval and gentamicin dose depend on age and birth weight; ceftriaxone cautions; oxygen target ≥90% (>94% with emergency signs).

- **Children ≥5 years**: fast breathing >30/min; UCG moderate vs severe categories; consider *Mycoplasma* (erythromycin; doxycycline only >8 y).
- **HIV-exposed or infected**: PCP (high-dose cotrimoxazole + prednisolone), TB, CMV, LIP; DNA PCR under 18 months; ART within 2 weeks.
- **Severe acute malnutrition**: signs blunted; antibiotics for all, careful fluids (no standard boluses), F-75, warmth, glucose (<3 mmol/L threshold); hypoglycaemia threshold higher.
- **Sickle cell disease**: risk factor for pneumonia Verify *acute chest syndrome management not covered in source; see sickle cell topic*.
- **Measles**: Staph pneumonia risk; vitamin A.
- **Cerebral palsy**: recurrent aspiration; add metronidazole.

14. Key Points

High-yield points

- **Count for a full minute**: 60–50–40 cut-offs.
- **Pulse oximeter on every child with cough or difficult breathing**; SpO_2 <90% = oxygen.
- Pneumonia (fast breathing and/or indrawing, no danger sign): oral amoxicillin 40 mg/kg twice daily for 5 days.
- Severe pneumonia: oxygen + ampicillin or benzylpenicillin + gentamicin for at least 5 days.
- Every infant under 2 months with pneumonia is severe.
- Not better at 48 h: think Staph, TB, empyema, PCP, heart failure, wrong diagnosis, foreign body.
- Bronchiolitis: supportive care; no routine antibiotics or steroids.
- HIV-exposed infant with severe hypoxaemia and few signs: add high-dose cotrimoxazole and prednisolone.
- Kerosene: never induce vomiting; observe 6 h.
- Test every febrile child for malaria; treat both if both are present.

Numbers & doses

Item	Value
Fast breathing	**≥60** (<2 m), **≥50** (2–11 m), **≥40** (1–4 y), **>30** (≥5 y, UCG)
Hypoxaemia	SpO_2 **<90%** (oxygen); target **≥94%** with emergency signs
Severe tachypnoea	RR **≥70**
Amoxicillin (pneumonia)	**40 mg/kg 12-hourly × 5 days** (3 days fast breathing only, low-HIV areas)
Amoxicillin by age (250 mg DT)	2–12 m: **1 tab**; 1–3 y: **2 tabs**; 3–5 y: **3 tabs** twice daily
Ampicillin	**50 mg/kg 6-hourly** IV/IM
Benzylpenicillin	**50,000 IU/kg 6-hourly**
Gentamicin	**7.5 mg/kg once daily** (5 mg/kg first week of life)
Ceftriaxone	**80 mg/kg once daily** (100 mg/kg sick infants)
Cloxacillin (Staph)	**50 mg/kg 6-hourly × ≥21 days**
Cotrimoxazole for PCP	**120 mg/kg/day** (TMP 20 mg/kg/day) in 3–4 doses × **21 days**
Erythromycin (pertussis)	**12.5 mg/kg 6-hourly** × 7 days (UCG; 14 classically)
Nasal prong flow	**0.5–1 L/min** neonates; **1–2 L/min** infants; **2–4 L/min** older
Wean oxygen	SpO_2 **≥90%** in air for **10–15 min**

Item	Value
Review	Outpatient pneumonia at **2–3 days**; inpatient failure at **48 h**
Hydrocarbon ingestion	Observe **≥6 h**
Hypoglycaemia	<2.5 mmol/L (<3 in SAM): **10% dextrose 5 mL/kg**
Transfuse	Hb **≤4 g/dL**, or 4–6 g/dL with distress, shock or impaired consciousness

Exam traps

- **"Crackles are needed to diagnose pneumonia."** No: WHO diagnosis is fast breathing and/or chest indrawing; auscultation is unreliable in infants.
- **"Chest indrawing = admit for IV antibiotics."** Since WHO 2014 (confirmed 2024), indrawing **without danger signs** can be treated with oral amoxicillin at home; admit if danger signs, SpO_2 <90%, SAM, age <2 months, or HIV-infected or unable to follow up.
- **"Cotrimoxazole for pneumonia."** No: amoxicillin is first line; cotrimoxazole is for PCP and prophylaxis.
- **"Salbutamol, steroids and antibiotics for bronchiolitis."** International guidance: supportive care only.
- **"Induce vomiting after kerosene ingestion."** Never.
- **"Ceftriaxone first for every admitted child."** First line is ampicillin or penicillin + gentamicin; ceftriaxone is second line (AWaRe Watch antibiotic).
- **"Normal SpO_2 means mild."** A child with danger signs is severe whatever the SpO_2.
- **"Fever + fast breathing = pneumonia, no need for an RDT."** Always test for malaria in Uganda.

Questions seniors ask

Viva questions

1. *What are the WHO fast-breathing cut-offs?* <2 months ≥60; 2–11 months ≥50; 12–59 months ≥40 breaths/min.
2. *Name the general danger signs.* Unable to drink or breastfeed, vomits everything, convulsions, lethargic or unconscious.
3. *What is the first-line treatment of severe pneumonia?* Oxygen if SpO_2 <90%, ampicillin 50 mg/kg 6-hourly (or benzylpenicillin 50,000 IU/kg) + gentamicin 7.5 mg/kg once daily, at least 5 days.
4. *Which organisms cause pneumatocoeles? Staphylococcus aureus* (also *Klebsiella*); treat with cloxacillin + gentamicin for 3 weeks.
5. *When do you suspect PCP?* HIV-exposed or infected infant 2–6 months, severe hypoxaemia with few signs, not on cotrimoxazole prophylaxis.
6. *How do you wean oxygen?* Daily trial off when stable; stop if SpO_2 ≥90% in air for 10–15 min, then recheck.
7. *What is the treatment of bronchiolitis?* Supportive: oxygen, feeds (NG), nasal suction; no routine antibiotics or steroids.

On the ward round

- **How do you classify this child?** State the age, RR against the cut-off, chest indrawing, danger signs and SpO_2, then the WHO class: no pneumonia, pneumonia, or severe pneumonia.
- **Why ampicillin and gentamicin, not ceftriaxone, first?** They cover pneumococcus, *H. influenzae* and Gram negatives effectively, are Access antibiotics, cheaper and reduce resistance; ceftriaxone is reserved for failure or very sick infants.
- **The child is not better at 48 hours. What do you think about?** Staph (pneumatocoeles, empyema), wrong diagnosis (TB, PCP, heart failure, foreign body), complications (effusion, abscess), malaria, anaemia or SAM, poor adherence or dosing; do a CXR, HIV test, Xpert and switch to ceftriaxone or cloxacillin as appropriate.

- **When do you stop oxygen?** When the child is clinically stable and SpO_2 stays ≥90% in room air for 10–15 minutes on a daily trial off; recheck after an hour and overnight.
- **How would you manage pertussis in a 6-week-old?** Admit, oxygen and suction for apnoea or cyanosis, small frequent or NG feeds, azithromycin 10 mg/kg once daily for 5 days, isolate, treat household contacts, complete DTP doses.
- **How does HIV change your management of severe pneumonia?** Add high-dose cotrimoxazole for PCP (21 days) with prednisolone if hypoxaemic in infants, consider TB, start or optimise ART, and continue cotrimoxazole prophylaxis.
- **What prevents pneumonia in Uganda?** Exclusive breastfeeding, PCV, Hib, pertussis and measles vaccines, good nutrition and zinc, clean cooking fuel, handwashing, HIV prevention and early ART, cotrimoxazole for HIV-exposed infants.

Mnemonics

- **Fast breathing "60–50–40"**: <2 months 60, 2–11 months 50, 1–5 years 40.
- **Danger signs "DVCL"**: can't **D**rink, **V**omits everything, **C**onvulsions, **L**ethargic or unconscious.
- **Hypoxaemia without an oximeter "CHIN-GRuNt"**: **C**yanosis, **H**ead nodding, **I**nability to feed, **N**asal flaring, **G**runting, **R**ate ≥70 (severe tachypnoea), u**N**conscious or lethargic.
- **Why is pneumonia not getting better? "STEP-HIV-F"**: **S**taph, **T**B, **E**mpyema, **P**CP, **H**eart failure, **I**nadequate dose or adherence, **V**irus or wrong diagnosis (bronchiolitis, asthma), **F**oreign body.
- **Pertussis stages "CPC"**: **C**atarrhal, **P**aroxysmal, **C**onvalescent.

OSCE tips

- **Counting RR**: expose the chest, child calm (on mother's lap, not feeding), watch the abdomen in infants, count a **full minute** with a timer, state the age cut-off out loud.
- **Chest indrawing**: say "the lower chest wall goes in when the child breathes in" and check the child is calm (not crying, not feeding).
- **Oxygen station**: choose prongs size, set the flow by age, fix prongs with tape, check SpO_2 after 15 min, explain weaning.
- **Counselling a mother on amoxicillin**: dose with the tablet in breast milk or water, twice daily for 5 days even if better, return signs, follow-up in 2–3 days.

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SELF-TEST

Flashcards

27 cards. Cover the right-hand column and test yourself.

#	Question	Answer
1	WHO/IMCI fast-breathing cut-offs by age.	**<2 months ≥60/min; 2–11 months ≥50/min; 12–59 months ≥40/min**; ≥5 years >30/min (UCG 2023). Count for a full 60 s in a calm child.
2	IMCI general danger signs.	**Unable to drink or breastfeed, vomits everything, convulsions, lethargic or unconscious.** Any one = severe disease → refer/admit.
3	WHO classification: 'Pneumonia' vs 'Severe pneumonia' (2–59 months).	**Pneumonia**: cough/DIB + fast breathing and/or chest indrawing, no danger sign → oral amoxicillin. **Severe**: + any danger sign, central cyanosis/**SpO_2 <90%**, or severe respiratory distress (grunting, very severe indrawing) → admit, oxygen, injectable antibiotics.
4	Amoxicillin for non-severe pneumonia: dose and duration (UCG 2023/WHO 2024).	**40 mg/kg twice daily (80 mg/kg/day) for 5 days.** WHO 2024: 3 days acceptable for fast breathing only in **low-HIV** areas; Uganda uses 5. DT 250 mg: 2–12 m 1 tab, 1–3 y 2 tabs, 3–5 y 3 tabs, twice daily.
5	First-line antibiotics for severe pneumonia (UCG 2023).	**Ampicillin 50 mg/kg IV/IM 6-hourly** (or **benzylpenicillin 50,000 IU/kg 6-hourly**) **+ gentamicin 7.5 mg/kg once daily**, at least 5 days (up to 10), then oral amoxicillin.
6	Second-line antibiotic if severe pneumonia is not better after 48 h.	**Ceftriaxone 80 mg/kg IV/IM once daily** (UCG 2023). Also look for Staph, empyema, TB, PCP, heart failure, foreign body.
7	Treatment of suspected staphylococcal pneumonia.	**Cloxacillin 50 mg/kg (25–50) IV/IM 6-hourly + gentamicin 7.5 mg/kg once daily** for **at least 21 days** (UCG). Clues: pneumatocoeles, empyema, pneumothorax, skin sepsis, after measles.
8	How are infants <2 months with pneumonia managed (UCG 2023)?	All treated as **severe**: admit, keep warm, breast milk/NGT, oxygen; **ampicillin 50 mg/kg (WHO: 12-hourly week 1, 8-hourly weeks 2–4, 6-hourly after; UCG 6-hourly) + gentamicin 5 mg/kg (first week; 3 mg/kg LBW) or 7.5 mg/kg once daily**; ceftriaxone 100 mg/kg if severely ill; ≥5 days (10 if sepsis, 21 if meningitis).
9	When is oxygen given (WHO/UCG) and what is the target?	**SpO_2 <90%** → oxygen to keep ≥90%. During resuscitation with emergency signs, target **≥94%**.
10	Signs predicting hypoxaemia when no oximeter is available.	**Central cyanosis, grunting, head nodding, nasal flaring, inability to drink/feed due to breathing, severe chest indrawing, RR ≥70, depressed consciousness** (WHO 2016; WHO 2024: RR ≥20 above the age cut-off).
11	Nasal prong oxygen flow rates by age.	**Neonates 0.5–1 L/min; infants 1–2 L/min; older children 2–4 L/min.** Face mask needs ≥5 L/min. Check SpO_2 3-hourly; wean when SpO_2 ≥90% in air for 10–15 min.
12	Define bronchiolitis (NICE NG9).	Child <2 years (peak 3–6 months) with **coryzal prodrome 1–3 days**, then **persistent cough** and **tachypnoea or recession**, and **wheeze or crackles**. Usually **RSV**. Peaks day 3–5; cough resolves in 90% by 3 weeks.
13	Bronchiolitis management (NICE NG9/WHO).	**Supportive**: oxygen if SpO_2 <90–92%, small frequent or **NG feeds**, gentle nasal suction, CPAP if failing. **No** routine antibiotics, steroids, salbutamol, adrenaline or hypertonic saline. (UCG 2023 allows a salbutamol trial, stop if no response.)
14	Severe bronchiolitis criteria (UCG/NICE).	**Age <3 months**, RR >60–70, poor feeding (<50–75% of normal), **SpO_2 <92%**, apnoea, grunting, cyanosis, exhaustion.

#	Question	Answer
15	Clinical stages of pertussis.	**Catarrhal** (1–2 wk, most infectious, coryza) → **Paroxysmal** (1–6 wk: paroxysms, whoop, post-tussive vomiting, subconjunctival haemorrhage; infants: **apnoea, cyanosis**, may not whoop) → **Convalescent** (weeks–months).
16	Antibiotic treatment of pertussis.	**Azithromycin**: <6 months 10 mg/kg once daily × 5 days; ≥6 months 10 mg/kg day 1 then 5 mg/kg days 2–5. Or **erythromycin 12.5 mg/kg 6-hourly** × 7 days (UCG; 14 classically). Avoid erythromycin <1 month (pyloric stenosis). Treat household contacts.
17	Drugs NOT to give in pertussis (UCG 2023).	**Cough mixtures, sedatives, mucolytics, antihistamines** — 'useless' and potentially harmful. Give oxygen, suction, small frequent feeds and a macrolide.
18	PCP treatment in an HIV-infected infant (UCG 2023).	**Cotrimoxazole 120 mg/kg/day** (TMP 20 mg/kg/day) in 3–4 doses × **21 days** + **prednisolone 2 mg/kg/day** × 5 days then taper (if hypoxaemic); oxygen; then cotrimoxazole prophylaxis; start ART.
19	Clinical clues to PCP.	HIV-exposed/infected infant **2–6 months**, not on cotrimoxazole, **severe hypoxaemia** with **few crackles**, fast breathing, afebrile or low fever; CXR diffuse interstitial infiltrates; high LDH.
20	Empyema: clinical signs and management.	**Persistent fever** on antibiotics, **stony dull percussion**, absent breath sounds, mediastinal shift. CXR/ultrasound, **diagnostic tap**, **chest drain**, cloxacillin + gentamicin (or ceftriaxone) for **3–4 weeks**; exclude TB.
21	Kerosene ingestion: key management points.	**No induced vomiting, no gastric lavage, no charcoal.** Oxygen if SpO_2 <90%, **observe ≥6 h**, CXR if symptomatic; antibiotics only for secondary bacterial pneumonia.
22	Back blows and thrusts for a choking infant vs child.	**Infant (<1 y)**: 5 back blows (head down along the forearm) + 5 **chest** thrusts. **Child >1 y**: 5 back blows + 5 **abdominal** thrusts. Unconscious → start CPR, look in the mouth before breaths.
23	Common causes of pneumonia by age (UCG 2023).	**Neonates**: GBS, Klebsiella, E. coli, S. aureus, Chlamydia. **<5 y**: **pneumococcus**, H. influenzae, S. aureus, RSV, influenza, measles. **≥5 y**: pneumococcus, Mycoplasma, viruses. **HIV**: Pneumocystis, TB, CMV.
24	Why is wheeze important when classifying pneumonia?	Wheeze + fast breathing may be **bronchospasm** (asthma, viral wheeze, bronchiolitis), not pneumonia. Give a rapid-acting bronchodilator and **re-count**; if fast breathing resolves, don't classify as pneumonia.
25	Reasons a child with pneumonia fails to improve (STEP-HIV-F).	**S**taph, **T**B, **E**mpyema/effusion, **P**CP, **H**eart failure, **I**nadequate dose/adherence, **V**iral/wrong diagnosis, **F**oreign body. Also malaria, anaemia, SAM.
26	Pneumonia burden (WHO).	**14% of under-five deaths: 740,180 children in 2019**; 22% of deaths at 1–5 y. Pneumonia + diarrhoea ≈ **23% of under-five deaths (1.17 million)** (WHO 2024 guideline).
27	Vaccines in UNEPI that prevent pneumonia.	**PCV** and **DTP-HepB-Hib** (pertussis, Hib) at **6, 10, 14 weeks**; **measles-rubella at 9 and 18 months**; BCG at birth (severe TB). Plus exclusive breastfeeding and cotrimoxazole for HIV-exposed infants.

SELF-TEST

Single best answer questions

15 questions. The answers and explanations follow at the end.

1. A 9-month-old boy (7.5 kg) has had cough and fever for 2 days. He is breastfeeding well and alert. RR 54/min (counted for 60 s while calm), no chest indrawing, no wheeze, SpO_2 96%. Malaria RDT negative. What is the correct classification and treatment?

- A No pneumonia: cough or cold; home care without antibiotics
- B Severe pneumonia; admit for ampicillin and gentamicin
- C Pneumonia; oral amoxicillin 250 mg twice daily for 5 days and review in 2–3 days
- D Bronchiolitis; nebulised salbutamol and discharge
- E Pneumonia; cotrimoxazole for 5 days

2. A 14-month-old girl (8.8 kg) has cough and fever for 3 days. Today she cannot drink. She is lethargic, RR 58/min, lower chest wall indrawing, grunting, SpO_2 87% in air. MUAC 12.8 cm. What is the BEST initial management?

- A Oxygen by nasal prongs, then IV ampicillin 440 mg 6-hourly plus gentamicin 66 mg once daily
- B Oral amoxicillin 500 mg twice daily at home and review in 2 days
- C IV ceftriaxone 2 g once daily as first line
- D Nebulised salbutamol and observe for 1 hour
- E IV fluids 20 mL/kg bolus, then oral amoxicillin

3. A 3-year-old boy (13 kg) with severe pneumonia has received ampicillin and gentamicin for 48 hours. He remains febrile at 39.4 °C with RR 55/min. Chest X-ray shows multiple thin-walled air-filled cysts in the right lung and a small right pleural effusion. He had multiple skin boils last week. What is the MOST appropriate change in antibiotics?

- A Continue ampicillin and gentamicin for another 5 days
- B Switch to oral amoxicillin
- C Add erythromycin for atypical pneumonia
- D Start high-dose cotrimoxazole for PCP
- E Switch to cloxacillin 650 mg 6-hourly plus gentamicin, continued for at least 3 weeks

4. A 4-month-old girl (5.5 kg) has had runny nose for 2 days, then cough and difficulty feeding. She is taking about half her usual breastfeeds. RR 62/min, mild subcostal recession, bilateral fine crackles and expiratory wheeze, T 37.8 °C, SpO_2 89% in air. What is the MOST appropriate management?

- A Oral amoxicillin and send home
- B Admit, oxygen by nasal prongs, NG feeds or small frequent breastfeeds, gentle nasal suction
- C Nebulised salbutamol every 20 minutes and oral prednisolone
- D IV ceftriaxone and dexamethasone
- E Chest physiotherapy every 4 hours

5. At a HC III with no pulse oximeter, which ONE finding in a 10-month-old with pneumonia MOST strongly suggests hypoxaemia and the need to give oxygen and refer?

- A Temperature 39 °C
- B Respiratory rate 52/min
- C Crepitations at the right base
- D Head nodding with each breath
- E Cough for 4 days

6. A 3-month-old HIV-exposed infant (4.6 kg) whose mother did not take ART is admitted with fast breathing (RR 72/min) and SpO_2 78% in air. Temperature 37.6 °C. The chest has only a few scattered crackles. He has never received cotrimoxazole. Which treatment should be added to ampicillin and gentamicin?

- **A** High-dose cotrimoxazole for 21 days plus prednisolone
- **B** Oral amoxicillin 3 days
- **C** Isoniazid preventive therapy
- **D** Fluconazole 6 mg/kg daily
- **E** Nebulised adrenaline

7. A 5-week-old girl (3.8 kg), whose DTP doses are not yet due, has had a runny nose for a week and now has bouts of coughing that end with her going blue and stopping breathing for 10–15 seconds, followed by vomiting. Between bouts she looks well and the chest is clear. Her 8-year-old brother has had a cough for 3 weeks. Which antibiotic is MOST appropriate for her? Advanced

- **A** Erythromycin 12.5 mg/kg 6-hourly for 14 days
- **B** Amoxicillin 40 mg/kg twice daily for 5 days
- **C** Azithromycin 10 mg/kg once daily for 5 days
- **D** Ceftriaxone 80 mg/kg once daily for 7 days
- **E** Cotrimoxazole prophylaxis

8. A 2-year-old boy (11 kg) drank kerosene from a soda bottle 30 minutes ago. He coughed and vomited once and now has RR 36/min, no indrawing, SpO_2 97%. His father asks you to make him vomit the rest. What is the BEST management?

- **A** Induce vomiting with salt water
- **B** Gastric lavage with normal saline
- **C** Give activated charcoal 1 g/kg
- **D** Start IV ceftriaxone to prevent pneumonia
- **E** Do not induce vomiting; observe for at least 6 hours with SpO_2 monitoring and CXR if symptoms develop

9. A 4-year-old girl (15 kg) has been treated for severe pneumonia for 4 days. She still has fever 39 °C, RR 48/min, and you find stony dull percussion with absent breath sounds over the left lower chest and the trachea deviated to the right. What is the MOST appropriate next step?

- **A** Add oral salbutamol
- **B** Double the dose of gentamicin
- **C** Discharge on oral amoxicillin
- **D** Chest X-ray/ultrasound and diagnostic pleural tap, then chest drain if pus
- **E** Start anti-TB treatment immediately without further tests

10. A 2-year-old boy (11 kg) with cough has RR 46/min and audible expiratory wheeze; he is alert and drinking, SpO_2 95%. What is the MOST appropriate next step before classifying him as pneumonia?

- **A** Start ampicillin and gentamicin
- **B** Give a rapid-acting bronchodilator (salbutamol via spacer) and re-count the respiratory rate after 15–20 minutes
- **C** Do a chest X-ray to confirm consolidation
- **D** Give prednisolone 2 mg/kg and send home
- **E** Classify as severe pneumonia because of wheeze

11. A 1-year-old (9 kg) on nasal prong oxygen at 1 L/min for severe pneumonia is now feeding, afebrile and comfortable, RR 38/min. Which is the CORRECT way to decide whether oxygen can be stopped?

A Stop oxygen when the course of antibiotics is finished
B Stop oxygen as soon as the child is afebrile
C Keep oxygen until discharge to be safe
D Increase to 2 L/min for 1 hour, then stop
E Do a trial off oxygen; stop if SpO_2 remains ≥90% in room air for 10–15 minutes, then recheck

12. A 3-year-old girl (14 kg) with cough has RR 38/min and lower chest wall indrawing. She is alert, drinking well, SpO_2 95%, no danger signs, HIV-negative, no malnutrition, and the family can return. According to the WHO 2024 guideline, what is the recommended treatment?

A Admit for IV benzylpenicillin and gentamicin for 5 days
B Oral amoxicillin 40 mg/kg twice daily for 5 days as an outpatient
C No antibiotic; she has no fast breathing
D Oral amoxicillin for 3 days only
E IM ceftriaxone once and review in a week

13. A 6-day-old term baby (3.1 kg) has fast breathing (RR 70/min), severe chest indrawing and poor feeding. Which antibiotic prescription is CORRECT? Advanced

A Oral amoxicillin 40 mg/kg twice daily at home
B Gentamicin 7.5 mg/kg IV once daily alone
C Chloramphenicol 25 mg/kg IV 6-hourly
D Ampicillin 50 mg/kg IV 12-hourly plus gentamicin 5 mg/kg IV once daily
E Cotrimoxazole 4 mg/kg twice daily

14. A 7-year-old boy (21 kg) has had cough, fever and night sweats for 5 weeks, with weight loss. His grandmother, who lives with him, is on TB treatment. He completed a course of amoxicillin 2 weeks ago without improvement. RR 30/min, SpO_2 95%. What is the MOST appropriate next step?

A A second course of amoxicillin for 10 days
B Admit for ceftriaxone
C Evaluate for TB: Xpert Ultra on sputum (or gastric aspirate/stool), chest X-ray and HIV test
D Start isoniazid preventive therapy
E Nebulised salbutamol and prednisolone

15. A 6-month-old (6.8 kg) with severe pneumonia is on IV fluids because she is too breathless to feed. Her serum sodium is 128 mmol/L and she is not dehydrated. What is the MOST appropriate fluid plan? Advanced

A Restrict fluids to about two-thirds of maintenance and start NG feeds when possible
B Give a 20 mL/kg bolus of normal saline
C Increase fluids to 150 mL/kg/day
D Give 3% saline 10 mL/kg over 10 minutes
E Stop all fluids for 24 hours

Answers & explanations

1. C · Pneumonia; oral amoxicillin 250 mg twice daily for 5 days and review in 2–3 days

C is right: at 2–11 months fast breathing is **≥50/min**; with no danger sign or hypoxaemia this is **pneumonia** → amoxicillin 40 mg/kg (one 250 mg DT) twice daily for 5 days (Uganda is a high-HIV setting) and review in 2–3 days. A is wrong: RR 54 is fast for his age. B is wrong: there are no danger signs, no hypoxaemia and no severe distress. D is wrong: there is no coryza-wheeze-crackles picture and salbutamol isn't standard for bronchiolitis. E is wrong: cotrimoxazole is no longer recommended for pneumonia; amoxicillin is first line.

2. A · Oxygen by nasal prongs, then IV ampicillin 440 mg 6-hourly plus gentamicin 66 mg once daily

A is right: a danger sign (lethargy, unable to drink) plus hypoxaemia = **severe pneumonia**: oxygen first, then **ampicillin 50 mg/kg (440 mg) 6-hourly + gentamicin 7.5 mg/kg (66 mg) once daily**. B is wrong: oral outpatient therapy is only for pneumonia without danger signs or hypoxaemia. C is wrong: ceftriaxone is second line, and 2 g is far above 80 mg/kg (≈700 mg) for her weight. D is wrong: there is no wheeze; salbutamol does not treat pneumonia. E is wrong: she is not in shock; boluses risk fluid overload, and oral therapy is inadequate for severe pneumonia.

3. E · Switch to cloxacillin 650 mg 6-hourly plus gentamicin, continued for at least 3 weeks

E is right: **pneumatocoeles + effusion + skin boils + failure of first line** = **staphylococcal pneumonia** → **cloxacillin 50 mg/kg (650 mg) 6-hourly + gentamicin** for ≥21 days (UCG 2023), and assess the effusion for empyema. A is wrong: ampicillin does not cover penicillinase-producing Staph, and he is failing. B is wrong: stepping down when worsening is inappropriate. C is wrong: Mycoplasma doesn't cause pneumatocoeles. D is wrong: nothing suggests HIV/PCP, and PCP causes interstitial changes, not cysts with effusion.

4. B · Admit, oxygen by nasal prongs, NG feeds or small frequent breastfeeds, gentle nasal suction

B is right: this is **severe bronchiolitis** (coryzal prodrome, crackles and wheeze, feeding about 50% of normal, SpO_2 <92%): **supportive care** with oxygen, feeding support (NG if needed) and suction. A is wrong: she is hypoxaemic and not feeding; she needs admission, and antibiotics are not routinely indicated. C is wrong: bronchodilators and steroids don't help bronchiolitis (NICE NG9; UCG allows only a salbutamol trial, stopped if no response). D is wrong: neither antibiotics nor steroids are indicated without evidence of bacterial infection. E is wrong: chest physiotherapy is not recommended in bronchiolitis.

5. D · Head nodding with each breath

D is right: **head nodding** (use of accessory neck muscles) is one of the clinical signs WHO uses to predict hypoxaemia when no oximeter is available (with central cyanosis, grunting, nasal flaring, inability to feed, RR ≥70). A is wrong: fever does not indicate hypoxaemia. B is wrong: RR 52 is fast for age but well below the severe tachypnoea threshold (≥70). C is wrong: crackles support pneumonia but do not predict hypoxaemia. E is wrong: cough duration is not a hypoxaemia sign.

6. A · High-dose cotrimoxazole for 21 days plus prednisolone

A is right: a 2–6-month-old **HIV-exposed infant** without prophylaxis with **severe hypoxaemia and few chest signs** suggests **PCP** → high-dose cotrimoxazole (UCG 120 mg/kg/day in divided doses for 21 days) with **prednisolone** for hypoxaemia, plus oxygen; test for HIV (DNA PCR) and start ART. B is wrong: oral amoxicillin is for non-severe pneumonia. C is wrong: TPT is for TB prevention after excluding active TB, not acute treatment. D is wrong: fluconazole treats candida/cryptococcus, not PCP. E is wrong: adrenaline is for croup, not pneumonia.

7. C · Azithromycin 10 mg/kg once daily for 5 days

C is right: paroxysms with **apnoea, cyanosis and post-tussive vomiting** in a young infant with a coughing sibling = **pertussis**. In infants under 1 month (and preferably under 6 months), **azithromycin 10 mg/kg once daily for 5 days** is preferred. She should be admitted for apnoea monitoring, oxygen and feeding, and household contacts treated. A is wrong: erythromycin in infants <1 month is linked to **infantile hypertrophic pyloric stenosis**; azithromycin is safer. B is wrong: amoxicillin is not active against Bordetella. D is wrong: ceftriaxone is not the treatment for pertussis. E is wrong: cotrimoxazole prophylaxis is for HIV-exposed infants and does not treat pertussis.

8. E · Do not induce vomiting; observe for at least 6 hours with SpO_2 monitoring and CXR if symptoms develop

E is right: hydrocarbons cause **chemical pneumonitis** when aspirated; management is supportive: **no emesis, no lavage**, observe ≥6 hours, oxygen if SpO_2 <90%, CXR if symptomatic. A is wrong: vomiting gives the kerosene a second chance to be aspirated. B is wrong: lavage also risks aspiration. C is wrong: charcoal does not bind hydrocarbons and provokes vomiting. D is wrong: prophylactic antibiotics are not recommended; treat only secondary bacterial pneumonia.

9. D · Chest X-ray/ultrasound and diagnostic pleural tap, then chest drain if pus

D is right: persistent fever with **stony dullness, absent breath sounds and mediastinal shift** = large **effusion/empyema**: image, tap (pus? Gram stain, culture, Xpert), and insert a **chest drain**, with antibiotics covering Staph for 3–4 weeks. A is wrong: there is no bronchospasm. B is wrong: doubling gentamicin risks toxicity and does not drain pus. C is wrong: she is getting worse. E is wrong: TB is in the differential of an effusion but must be investigated (tap with Xpert, lymphocytic exudate); acute post-pneumonic empyema is more likely here.

10. B · Give a rapid-acting bronchodilator (salbutamol via spacer) and re-count the respiratory rate after 15–20 minutes

B is right: in a wheezing child, fast breathing may be due to **bronchospasm**; WHO/IMCI advise a **bronchodilator trial** and re-assessment before classifying as pneumonia. A is wrong: he has no danger signs or hypoxaemia; injectable antibiotics are not indicated. C is wrong: CXR is not required to diagnose non-severe pneumonia and won't change the immediate step. D is wrong: steroids before a diagnosis (asthma vs viral wheeze vs pneumonia) is premature. E is wrong: wheeze alone is not a severity criterion.

11. E · Do a trial off oxygen; stop if SpO_2 remains ≥90% in room air for 10–15 minutes, then recheck

E is right: WHO advises a **daily trial off oxygen** when stable; discontinue if SpO_2 stays **≥90% in room air** for 10–15 minutes, and recheck after an hour and periodically for the first day. A is wrong: oxygen need is unrelated to antibiotic duration. B is wrong: fever does not determine oxygenation. C is wrong: unnecessary oxygen wastes a scarce resource that other children need. D is wrong: increasing flow before stopping has no rationale.

12. B · Oral amoxicillin 40 mg/kg twice daily for 5 days as an outpatient

B is right: WHO (2014, confirmed in the **2024 guideline**) recommends **oral amoxicillin for 5 days as an outpatient** for children 2–59 months with **chest indrawing without danger signs**, rather than injectable antibiotics in hospital. A is wrong: inpatient injectable therapy is for severe pneumonia (danger signs, hypoxaemia). C is wrong: chest indrawing alone classifies as pneumonia. D is wrong: the 3-day option applies only to fast-breathing pneumonia without indrawing, in low-HIV settings. E is wrong: a single ceftriaxone dose is not a recommended regimen and uses a Watch antibiotic.

13. D · Ampicillin 50 mg/kg IV 12-hourly plus gentamicin 5 mg/kg IV once daily

D is right: an infant <2 months with pneumonia is treated as **severe**; in the **first week of life** ampicillin is given 12-hourly (50 mg/kg) and **gentamicin 5 mg/kg once daily** (UCG 2023). A is wrong: oral outpatient therapy is not appropriate for a neonate with severe signs. B is wrong: 7.5 mg/kg is the dose after the first week, and gentamicin alone misses GBS and Listeria cover. C is wrong: chloramphenicol is contraindicated in neonates <7 days (grey baby syndrome). E is wrong: cotrimoxazole is not used for neonatal pneumonia and is avoided in the first weeks (kernicterus risk).

14. C · Evaluate for TB: Xpert Ultra on sputum (or gastric aspirate/stool), chest X-ray and HIV test

C is right: cough and fever **>2 weeks**, night sweats, **weight loss**, a **household TB contact** and failure of antibiotics = presumptive **TB**; request **Xpert MTB/RIF Ultra** (sputum, induced sputum, gastric aspirate or stool), CXR and an HIV test. A is wrong: repeating antibiotics delays the diagnosis. B is wrong: he is not severely ill and the problem is not bacterial pneumonia. D is wrong: TPT is only given after active TB has been excluded; giving isoniazid alone to a child with TB disease breeds resistance. E is wrong: there is no evidence of asthma.

15. A · Restrict fluids to about two-thirds of maintenance and start NG feeds when possible

A is right: hyponatraemia without dehydration in severe pneumonia suggests **SIADH**; restrict to about **two-thirds of maintenance**, use isotonic fluids and move to NG/breast milk early. B is wrong: she is not shocked or dehydrated; a bolus worsens overload. C is wrong: high volumes worsen hyponatraemia and pulmonary oedema. D is wrong: hypertonic saline is reserved for symptomatic severe hyponatraemia (seizures), not a stable infant at 128. E is wrong: stopping all fluids risks hypoglycaemia and dehydration.

SELF-TEST

Clinical cases

Work through each step before reading the model answer.

Case 1: Severe pneumonia in an HIV-exposed infant

Namubiru, a 10-month-old girl from Nansana, is brought to Mengo Hospital with 3 days of cough and fever. Today she refused the breast. Her mother is HIV-positive on TLD; Namubiru had a negative DNA PCR at 6 weeks and is still breastfeeding.

1. What are your first actions in the emergency room?

> ETAT: check airway/breathing (cyanosis, severe distress), circulation, consciousness; **SpO_2**; weigh; RBS; malaria RDT. Give **oxygen** if SpO_2 <90% or emergency signs; position on mother's lap; gentle suction.

2. Findings: 7.2 kg (WHZ –1.5, MUAC 12.4 cm, no oedema), lethargic, RR 64, lower chest wall indrawing, grunting, SpO_2 86%, T 39 °C, bronchial breathing right upper zone, RDT negative, RBS 4.8. Classify.

> **Severe pneumonia with hypoxaemia** (danger signs: lethargy and unable to breastfeed; SpO_2 <90%; grunting).

3. Write the admission orders.

> 1. **Oxygen** by nasal prongs 1 L/min, target SpO_2 ≥90% (≥94% initially if emergency signs).
> 2. **Ampicillin 360 mg IV 6-hourly** (50 mg/kg) + **gentamicin 54 mg IV once daily** (7.5 mg/kg).
> 3. **Paracetamol 108 mg** (15 mg/kg) 6-hourly prn.
> 4. **NG expressed breast milk** (avoid IV fluids if possible); maintenance about 100 mL/kg/day.
> 5. Monitor RR 2-hourly, SpO_2 3-hourly, temperature 6-hourly, feeding, alertness.

4. What about her HIV status?

> She is **HIV-exposed and still breastfeeding**, so a negative 6-week PCR does not exclude infection. Do a repeat test now (rapid antibody at ≥9 months; DNA PCR if reactive). If positive or severe hypoxaemia without response: add **high-dose cotrimoxazole + prednisolone** for PCP, start ART. Ensure cotrimoxazole prophylaxis continues until HIV is excluded after breastfeeding ends.

5. At 48 hours she is afebrile, feeding by breast, RR 44, SpO_2 93% on 0.5 L/min. Next steps?

> Improving: continue IV antibiotics to complete ≥3 days then switch to **oral amoxicillin 40 mg/kg (≈300 mg) twice daily** to complete at least 5 days total; **daily trial off oxygen** (stop if SpO_2 ≥90% in air for 10–15 min); feeding counselling.

6. What must happen before discharge?

> Off oxygen ≥24 h, feeding well; complete antibiotics; **HIV result** and linkage; **catch-up vaccines** (MR1 given? PCV complete?); nutritional follow-up (WHZ –1.5); counselling on danger signs, smoke and breastfeeding; review in 1 week.

Take-away: Danger sign plus hypoxaemia = severe pneumonia: oxygen first, then ampicillin + gentamicin by weight, careful feeding, and never leave an HIV-exposed infant's status unresolved.

Case 2: The child who did not get better

Okello, a 3-year-old boy (13 kg) from Gulu, was admitted 2 days ago with severe pneumonia and is on ampicillin and gentamicin. He had measles 3 weeks ago. The nurse reports he is still febrile and breathing fast.

1. List the reasons a child may fail first-line treatment.

> **STEP-HIV-F**: Staph (especially after measles), TB, Empyema/effusion, PCP (HIV), Heart failure, Inadequate dosing/adherence, Viral or wrong diagnosis, Foreign body; plus malaria, severe anaemia, SAM.

2. Examination: T 39.6 °C, RR 56, SpO_2 89% on 2 L/min, stony dullness and absent breath sounds right base. CXR: right effusion and two left-sided pneumatocoeles. Interpretation?

> **Staphylococcal pneumonia with empyema** (post-measles, pneumatocoeles, effusion, failure of ampicillin).

3. What procedures and antibiotics do you order?

> Ultrasound-guided **diagnostic tap** (pus → Gram stain, culture, Xpert, pH) and **chest drain** with underwater seal. Switch to **cloxacillin 650 mg IV 6-hourly** (50 mg/kg) **+ gentamicin 98 mg once daily** (7.5 mg/kg), for **≥3 weeks** (IV until afebrile, then oral). Increase oxygen; analgesia.

4. Two days later he suddenly becomes more breathless, SpO_2 falls to 80%, and the right chest is hyper-resonant. What has happened?

> **Pyopneumothorax** (ruptured pneumatocoele or bronchopleural fistula). Check the drain (blocked? displaced?), needle decompression if tension, reposition/insert a new drain, high-flow oxygen, senior surgical review.

5. What else should you address because of his measles?

> **Vitamin A** (if not given: 200,000 IU for ≥12 months, on 2 consecutive days), nutritional assessment (measles → malnutrition), HIV test, check eyes, and **catch-up vaccines** (MR2, PCV) after recovery.

Take-away: A child not better at 48 hours is a diagnostic problem: look for Staph (post-measles, pneumatocoeles), empyema, TB and HIV, and change both the antibiotic and the plan.

Case 3: Apnoea in a young infant

Baby Ruth, 6 weeks old (4.0 kg), is brought to a HC IV because she 'stops breathing and turns blue when she coughs'. She had a runny nose 10 days ago. Her 9-year-old sister has had a cough for 3 weeks that makes her vomit. Ruth received BCG and OPV0 at birth; her 6-week vaccines are due today.

1. What is the most likely diagnosis and what supports it?

> **Pertussis**: catarrhal prodrome, then paroxysms with **apnoea and cyanosis** (young infants may not whoop), a sibling with a prolonged cough and post-tussive vomiting, and no DTP yet.

2. What are your immediate management priorities?

> **Admit** (age <6 months with apnoea). Oxygen if SpO_2 <90% or cyanosis; **gentle suction** after paroxysms; nurse near the nursing station with an apnoea/SpO_2 monitor; **small frequent breastfeeds**, NG feeds if needed; keep warm; check glucose.

3. Which antibiotic and dose?

> **Azithromycin 10 mg/kg = 40 mg orally once daily for 5 days** (preferred under 1 month and safe at 6 weeks). Erythromycin 12.5 mg/kg 6-hourly is the UCG option but is linked to pyloric stenosis in young infants. Isolate until 5 days of treatment.

4. What will you NOT give, and why?

> No **cough mixtures, sedatives, mucolytics or antihistamines** (UCG: useless; sedatives worsen apnoea).

5. What do you do for the family and the community?

> **Treat the sister and household contacts** with a macrolide; give Ruth her **6-week vaccines** (pentavalent, PCV, OPV, rotavirus) once stable; check the sister's immunisation card; **notify** the district surveillance focal person (pertussis is notifiable); explain that the cough may last up to 3 months.

Take-away: Young infants with pertussis present with apnoea rather than a whoop; admit, support breathing and feeding, give azithromycin, and protect the household with prophylaxis and vaccination.

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A study aid, not a protocol. Check doses against the Uganda Clinical Guidelines 2023, your unit's protocol and a senior before you treat a patient.